NUR258SCE Septic Shock Simulation Scenario

(Revised 1/6/2026 CA)

Client: Rou Ann Young

Discipline: Adult Health Nursing II	**Student Level:** Advanced Med/Tele
Expected Simulation Run Time: 5 hours	**Location for Reflection:** Joyce Briefing Center
Location: Joyce Simulation Center	**File Name:** NUR258SCE. Septic Shock. NLN

Brief Description of Client

Hospital Location: Medical/Telemetry Unit **Shift:** Day shift

Name: Rou Ann Young

Date of Birth: 1-18-19XX	**Age:** 37	**Allergies:** Sulfa
Admit Weight: 84.1kg	**Height**: 167cm	**Pronouns:** She/her
Marital Status: Married	**Language**: English	**Sex:** Female

Attending Provider/Team: Maria Lester, MD

Primary Admitting Medical Diagnosis: Appendicitis, Open appendectomy

Past Medical History: Obesity, Hypertension, Diabetes Type 2

Current Hospital Stay: The client is a 37-year-old female, postoperative day four following an open appendectomy for a ruptured appendix. She has remained hospitalized on the surgical unit. During the night shift, the client reported increased pain over the past two hours and feeling chilled. You are assuming care at the start of the day shift.

Instructor Synopsis: The scenario will begin with the client already identified as septic. At the start of the scenario, students should be informed that the client meets sepsis criteria and that the provider is entering sepsis protocol orders, which will be available once the scenario begins. Students are expected to prioritize and carry out the provider orders in the appropriate sequence based on the client's clinical cues. Students should be informed that blood cultures, an ABG, lactic acid, and a urinalysis have already been obtained and do not need to be collected during the scenario. The client may display intermittent crying but will be primarily lethargic.

The objective of this scenario is for students to practice prioritization of care for a client with sepsis.

Simulation Learning Objectives

NUR258 Course Outcomes

1. Use critical thinking, clinical reasoning, and clinical judgement to prioritize comprehensive, individualized, and holistic care of the adult client. (CO1)
2. Select appropriate priorities for nursing assessment and interventions when delivering safe nursing care for adults. (CO2)
3. Deliver evidence-based holistic nursing care for adult clients with health alterations. (CO3)
4. Evaluate outcomes of nursing care of adult clients with health alterations. (CO4)

Simulation Scenario Objectives

1. Use critical thinking to recognize at least two cues of Shock or MODS. (CO 1)
2. Based on recognized cues, perform appropriate interventions for the recognized cues. (CO 2)
3. After taking action and performing interventions, reassess the client to ascertain if the performed interventions were successful. (CO 4)

Faculty Reference

The Healthcare Simulation Standards of Best Practice™
https://www.inacsl.org/healthcare-simulation-standards

Setting/Environment

Medical/Telemetry Unit

Equipment/Supplies Septic Shock: Rou Ann Young

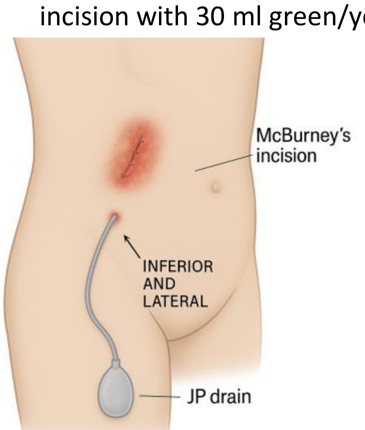
Simulated Patient/Manikin(s) Needed: HAL
Recommended Mode for Simulator: preprogramed manual advance
Other Props & Moulage:

Equipment Attached to Manikin/Simulated Client:

- ID band for client
- Female genitalia
- IV left forearm
- Alaris programed IV pump at 0.9% NS 30mL per hour. 2 channels on pump
- Foley catheter (with a port) in place, 30 ml amber/cloudy/sediments urine in bag. Foley secured with STAT lock.

Moulage and manikin programming:

- Incision lower quadrant 8cm long. Surgical staples, spaced 1 cm apart.
- Wound edges, poorly approximated, with mild dehiscence (opening of wound) at the lower end.
- JP Drain moulage: JP drain located inferior-lateral to incision with 30 ml green/yellow, cloudy drainage.



Equipment Available in Room:

- Equipment to take vitals: Pulse oximeter, telemetry cables (3 lead), thermometer.
- 02 delivery device: nasal cannula and simple mask.
- Toothette or toothbrush
- Bedpan/urinal
- Black bin to deposit used supplies **<u>(at least 2 bins)</u>**
- Emesis basin
- Water cup and drinking straw.
- Suction tubing and suction canister.
- Incentive spirometer
- Grip socks
- Primary IV tubing **<u>(at least 12 sets)</u>**

Additional Equipment Available for Scenario:

- Sequential Compression Device (SCD)

<u>Scenario Programming</u>

<u>Vitals</u>

HR: 135	**BP**: 98/52	**Temp**: 38.1
RR: 28	**SpO2**: 85% Room Air	

All Medications and fluids needed for scenario (and quantity of medication)	
Oral Medications Subcutaneous • Regular Insulin 10 units, (1 vial) Green: Need in SimServe Yellow: in the room, running	IV Injections • Ondansetron 4 mg/2 mL (1 vial) IV Drip • 0.9% NACL 30 ml/hr. (1 bag) • Acetaminophen 1000 mg/100 ml (1 bag) • Lactate Ringers 30 ml/kg, (2 bags) • Vancomycin 1 gram/ 250 ml in 0.9% NACL, every 12 hours, IV drip. (1bag)

Roles

Students	Simulation Staff
Lead Nurse	Nursing Instructor & SOS/Tech Team acting as following: • Physician • Charge Nurse • Pharmacy • Radiology • Cardiac/EKG Tech • Phlebotomy/Lab • Operator • Patient

Guidelines/Information Related to Roles

All- Employ the TeamSTEPPS Principles of leadership, team structure, communications, situation monitoring & mutual support. Monitor patients' condition and intervene as appropriate.

Lead Nurse- Responsible for PT outcome & team performance. Conduct briefing assign tasks & set common goals & objectives. Patient assessment. Direct the team in an efficient & effective manner to improve the patient's condition. Communicate with the team and MD.

Pre-briefing/Briefing

1. Priorities based on client cues.
2. Possible organ dysfunction and cues.
3. Physical assessment and client cues.
4. Expected and concerning lab results.
5. Expected and concerning diagnostic results.
6. Forming a hypothesis and making recommendations based on client cues.
7. Expected medications for client.
8. SIRS protocol
9. Sepsis vs septic shock

Student Shift Report: **verbally** given to students by instructor.

(Please do not give the students a physical copy of this.)

Client Name	**Code Status**	**Allergies**	**DOB**	**Rm #**	**MD**
Rou Ann Young	FULL	Sulfa	1/18/19XX **Age** 37	211	Maria Lester, MD

Vital Signs **HR** 98 **O2** 93% Room Air **BP** 132/86 **Temp** 39.2 C **RR** 24	**Home Medications** • Metformin 1000 mg BID • Lisinopril 20 mg daily • Trazodone 200 mg daily • Famotidine 20 mg daily **Health History** • Hypertension • Diabetes Type 2 • Obesity

Admitting Diagnosis: 4 days Post-Op Open Appendectomy d/t ruptured appendix

The client is a 37-year-old female who is postoperative day four following an open appendectomy for a ruptured appendix. She has been recovering on the medical-telemetry unit without complications and has been participating in daily physical and occupational therapy. Her husband is currently at work and plans to visit later this evening.

Based on abnormal vital signs, the client met SIRS criteria. The provider was notified, and the sepsis protocol was initiated. An arterial blood gas (ABG), blood cultures, lactic acid level, and urinalysis with culture if indicated were obtained, and results have now returned. Based on these findings, the client is currently flagged as septic. A Foley catheter has been inserted, and the client's diet has been changed to NPO per orders. The rest of the orders need to be completed.

Neuro Anxiety, pain Follows commands	**Cardiology** NSR S1/S2
Respiratory Clear, equal, regular	**GI** Hypoactive Soft, tender surgical sites
Skin/Wounds Incision right lower quadrant at McBurney's point ~ 8cm long. Surgical staples, spaced 1 cm apart. Clean/dry/intact	**GU** Continent Foley in place
Drains/IVs 18g IV Left Forearm. Running 0.9% NACL 30 ml/hr. continuously R JP drain is purulent.	**Diagnostics** WNL
Pain 7/10	**Fall Risk** High due to poor mobility Uses call light appropriately.
Psycho/Social Married, husband is at work	**Diet** NPO Blood glucose checks every 6 hours.

ATI: INITIAL ORDERS (Everything listed below will already be visible to the student in ATI when the scenario is started.)

Medications	Labs/Diagnostics	Nursing/Misc. orders
• Ondansetron 4 mg IV push PRN every 6 hours. • Acetaminophen 1000 mg/100 ml IV drip, PRN Q8 for fever >38. Infuse over 15 min. • Lactate Ringers 30 ml/kg, per the sepsis resuscitation protocol. Notify provider of any significant changes in patient's LOC or Vital signs. • D/C 0.9% NaCl • Vancomycin 1 gram/ 250 ml in 0.9% NACL, every 12 hours, IVPB infuse over 60 minutes. • Regular Insulin 10 units, SQ, recheck blood glucose in 30 minutes.	• Stat ABG • Blood cultures X2 • Stat CBC • Stat CMP • Stat Lactate • Urinalysis • Stat Chest x-ray • Stat abdominal/pelvis CT	• NPO • POC blood glucose monitoring: fingerstick blood glucose checks. Notify provider if BG is >300. • Please take initial vital signs, then monitor per unit protocol. Please notify provider for any vitals outside of normal parameters. • Maintain O2 sat greater than 94% • Continuous pulse-ox • Insert indwelling foley catheter • Vitals q15 minutes until hemodynamically stable

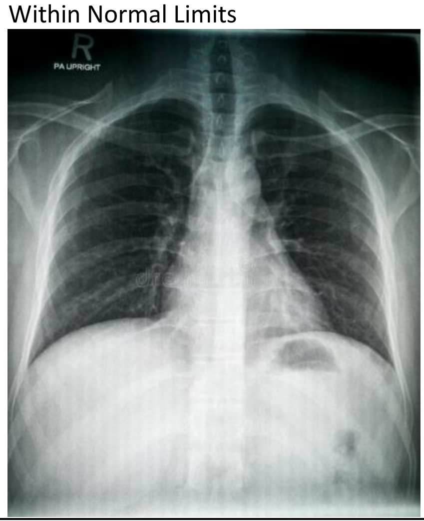
Scenario Labs/Diagnostics labs viewable at beginning of scenario.

Surgery Day			
CMP & CBC	**Chemistry/ Metabolic**		
	Na	140	135-145
	K	4.2	3.5-5
	Cl	98	98-106
	CO2	27	23-29
	BUN	16	10-20
	Creatinine	1.0	0.6-1.2
	Glucose	249	60-100
	CBC		
	WBC	8	5-10
	Hgb	15	14-18
	Hct	48	42-52%
	Platelets	328	150-400
Post-Op Day 1			
CMP & CBC	**Chemistry/ Metabolic**		
	Na	138	135-145
	K	4.8	3.5-5
	Cl	100	98-106
	CO2	24	23-29
	BUN	11	10-20
	Creatinine	1.2	0.6-1.2
	Glucose	190	60-100
	CBC		
	WBC	8	5-10
	Hgb	13	14-18
	Hct	41	42-52%
	Platelets	198	150-400

Post-Op Day 2	
CMP & CBC	**Chemistry/ Metabolic** Na 142 135-145 K 3.9 3.5-5 Cl 98 98-106 CO2 27 23-29 BUN 16 10-20 Creatinine 1.0 0.6-1.2 Glucose 201 60-100 **CBC** WBC 8 5-10 Hgb 15 14-18 Hct 48 42-52% Platelets 328 150-400
Post-Op Day 3	
CMP & CBC	**Chemistry/ Metabolic** Na 140 135-145 K 4.2 3.5-5 Cl 98 98-106 CO2 27 23-29 BUN 16 10-20 Creatinine 1.0 0.6-1.2 Glucose 249 60-100 **CBC** WBC 11 5-10 Hgb 15 14-18 Hct 48 42-52% Platelets 328 150-400
Day of Scenario LABS: Post-Op Day 4	
CMP & CBC	**Chemistry/ Metabolic** Na 134 135-145 K 4.8 3.5-5 Cl 99 98-106 CO2 28 23-29 BUN 18 10-20 Creatinine 1.0 6-1.2 Glucose 348 60-100 Lactic Acid 3.1 0.5-2.2 **CBC** WBC 21 5-10 Hgb 8 14-18 Hct 23 42-52% Platelets 220 150-400
ABG	**ABG** PH 7.31 7.35-7.45 PaCO2 35 35-45 PaO2 90 80-100 HCO3 19 22-26

Urinalysis	**Cath UA**		
	Color	Amber	Yellow
	Appearance	Cloudy/sediments	Clear
	PH	6.1	5-7
	Blood	1+	none
	Protein	1+	none
	Glu	2+	none
	Leukocyte	Esterase	none
	Nitrite	none	none
	RBC	1	0-5
	WBC	2	0-3
	Bacteria	trace	none
	Mucous	present	none

Scenario Labs/Diagnostics labs to be pushed out during the scenario.

Chest X-Ray	Within Normal Limits 
CT abd/pel w/ IV contrast	**<u>Abdominal/Pelvis CT</u>** A 4.2 cm x 3.8 cm hypodense collection is seen in the right lower quadrant, adjacent to the cecum and surgical site. This is consistent with a localized intra-abdominal abscess.

Scenario Progression Outline

Client Name: Rou Ann Young
Date of Birth: 1/18/19XX
Doctor Name: Maria Lester, MD
Allergies: Sulfa

*****Instructor need to release Sepsis order PRIOR to beginning scenario. *****

Phase	Manikin/SP Actions	Expected Nursing Interventions	MD Orders and Student Cues
Team 1	**Vitals** **HR**: 135 **BP:** 98/52 **RR:** 28 **SpO2:** 85% **Temp:** 38.1 **Pain:** 7/10 • **Neuro**: Lethargic • **Cardiac**: Tachycardia • **Pulses**: thready/weak • **Capillary Refill**: sluggish • **Abdomen:** Tenderness around incision site, client is starting to feel nauseous. **Vital Sign Changes when fluid is administered, and Oxygen is placed.** **HR**: 120 **BP:** 105/68 **RR:** 20 **SpO2:** 95% (4L NC) **Temp:** 37.8 **Pain:** 3/10	**Prior to calling the MD the students should do the following:** ❑ Obtain vital signs (BP, HR, O2, Temp, RR). ❑ Perform assessment. ❑ Assess JP drains. ❑ Obtain blood glucose. **Result: 350** ❑ Administer Oxygen ❑ Administer Fluids. ❑ Administer antibiotic. ❑ Administer IV Tylenol ❑ OPTIONAL: administer ondansetron ***The students should recognize the urgency of the fluid bolus over the antibiotic based on vital signs.*** Scenario Objectives: 1. Use critical thinking to recognize two cues of shock or MODs. (CO 1) ❑ Abnormal v/s ❑ Abnormal lab results ❑ Decreased urine output ❑ Change in mental status 2. Based on recognized cues, perform appropriate interventions for the recognized cues. (CO 2) ❑ Administer fluids ❑ Administer oxygen ❑ Administer antibiotic ❑ Administer Tylenol IV 3. After taking action and performing interventions, reassess the client to ascertain if the performed interventions were successful. (CO 4) ❑ Reassess client after administering fluids ❑ Reassess client after administer oxygen ❑ Reassess v/s	**Student Cues** • Some crying, but mostly lethargic. • Pain 7/10, abdomen • Nauseated **Call to MD:** • No new orders. The students need to complete all the orders prior to calling the MD. If a call is made, it will just be an update for the MD. • If the student asks about the results of CT and x-ray, can push the results out. Results will not be available at the beginning of the scenario.

Debriefing/Guided Reflection

Debriefing Phase	Debriefing Questions for Consideration
Reactions/ Defuse	How did you feel throughout the simulation experience?
	Give a brief summary of this patient and what happened in the simulation.
	What were the main problems that you identified?
Analysis/ Discovery	Discuss the knowledge guiding your thinking surrounding these main problems.
	What were the key assessment and interventions for this patient?
	Discuss how you identified these key assessments and interventions.
	Discuss the information resources you used to assess this patient. How did this guide your care planning?
	Discuss the clinical manifestations evidenced during your assessment. How would you explain these manifestations?
	Explain the nursing management considerations for this patient. Discuss the knowledge guiding your thinking.
	What information and information management tools did you use to monitor this patient's outcomes? Explain your thinking.
	How did you communicate with the patient?
	What specific issues would you want to take into consideration to provide for this patient's unique care needs?
	Discuss the safety issues you considered when implementing care for this patient.
	What measures did you implement to ensure safe patient care?
	What other members of the care team should you consider important to achieving good care outcomes?
	How would you assess the quality of care provided?
	What could you do improve the quality of care for this patient?
Summary/ Application	If you were able to do this again, how would you handle the situation differently?
	What did you learn from this experience?
	How will you apply what you learned today to your clinical practice?
	Is there anything else you would like to discuss?

SEPTIC SHOCK Rou Ann Young

Section	Content
Scenario Name	Septic Shock – Post-Operative Infection (Rou Ann Young)
Setting	Medical-Telemetry Unit Post-Operative Patient
Primary Learning Focus	Recognition and management of septic shock with emphasis on early intervention prioritization and reassessment
Priority Hypothesis	Septic shock secondary to intra-abdominal infection (post-op abscess) leading to poor perfusion tissue hypoxia and early organ dysfunction
Secondary Hypotheses	Hypovolemia due to sepsis hyperglycemia contributing to infection progression early MODS development
Key Supporting Cues	Tachycardia hypotension fever tachypnea hypoxia lethargy elevated WBC elevated lactate hyperglycemia decreased urine output purulent JP drainage
Red Flags / Escalation Triggers	BP <90 systolic or MAP <65 SpO2 <94 elevated lactate altered LOC decreased urine output persistent tachycardia signs of poor perfusion
Expected Clinical Progression if Missed	Sepsis → septic shock → MODS → organ failure → death
Common Student Errors	Delaying fluids prioritizing antibiotics first missing hypotension significance not reassessing after fluids focusing on labs over clinical picture delayed oxygen administration
Clinical Judgment Focus	Early recognition of shock prioritization of fluids over antibiotics linking vitals and labs to perfusion reassessment after intervention
Real-World Translation	This patient would trigger sepsis protocol rapid fluid bolus broad-spectrum antibiotics continuous monitoring possible ICU transfer
Instructor Teaching Emphasis	Fluids first then antibiotics trend vital signs lactate interpretation reassessment after every intervention recognition of shock before labs confirm

SEPTIC SHOCK Rou Ann Young

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Scenario Overview	Post-op day 4 ruptured appendix	High risk for intra-abdominal infection and sepsis	Immediate	Source of infection leading to shock	Progression to septic shock	Recognize infection risk and monitor closely	Ongoing	B	Post-op infections are high risk	✓	Yes
Scenario Overview	Sepsis protocol initiated	Patient already identified as septic	Immediate	Indicates urgency and need for rapid intervention	Delayed treatment leading to shock	Follow full protocol immediately	Ongoing	A	Sepsis is time-sensitive	✓	Yes
Baseline	HR 98 baseline	Normal baseline HR	Relevant	Used for comparison	Miss trend	Trend vitals	Ongoing	A	Trends matter		No
Baseline	Temp 39.2	Fever indicates infection	Important	Key infection indicator	Worsening infection	Administer antipyretics	Reassess temp	B	Infection recognition	✓	Yes
Baseline	RR 24	Tachypnea early sign of sepsis	Immediate	Early compensation for acidosis	Respiratory failure	Monitor closely	Trend RR	A	RR early indicator	✓	Yes
Baseline	O2 93%	Borderline oxygenation	Important	Risk for decline	Hypoxia	Apply O2 if needed	Monitor	A	Early hypoxia	✓	Yes
Baseline	BP 132/86	Normal baseline perfusion	Relevant	Comparison point	Miss decline	Trend BP	Ongoing	A	Trend awareness		No
Baseline	WBC rising to 21	Severe infection response	Important	Supports sepsis diagnosis	Miss severity	Monitor labs	Trend	B	Labs support picture	✓	Yes
Baseline	Lactate 3.1	Tissue hypoperfusion marker	Immediate	Key sepsis severity indicator	Organ failure	Act urgently with fluids	Reassess lactate	A	Perfusion marker	✓	Yes
Baseline	Glucose 348	Hyperglycemia in stress/sepsis	Important	Worsens outcomes	Delayed healing	Administer insulin	Reassess glucose	B	Glucose control	✓	Yes

SEPTIC SHOCK Rou Ann Young

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Baseline	Incision dehiscence	Poor wound healing infection source	Important	Source of sepsis	Spread infection	Assess wound	Ongoing	B	Source control		Yes
Baseline	JP drain purulent drainage	Active infection	Immediate	Direct infection evidence	Sepsis progression	Monitor drainage	Ongoing	B	Infection cue	✓	Yes
Baseline	Foley cloudy urine	Possible infection or poor perfusion	Important	GU involvement	Miss infection	Monitor output	Ongoing	B	Urine clues	✓	Yes
Baseline	NPO status	Prepares for procedures	Relevant	Not priority	Minimal	Maintain NPO	N/A	B	Context		No
Baseline	Anxiety and lethargy	Change in mental status	Immediate	Early organ dysfunction	Miss deterioration	Assess LOC	Frequent	A	LOC changes	✓	Yes
Phase 1 Deterioration	HR 135	Tachycardia from shock	Immediate	Compensatory mechanism	Shock progression	Monitor closely	Trend	A	Compensation	✓	Yes
Phase 1 Deterioration	BP 98/52	Hypotension	Immediate	Indicates poor perfusion	Organ failure	Start fluids immediately	Reassess BP	A R	Shock recognition	✓	Yes
Phase 1 Deterioration	RR 28	Tachypnea	Immediate	Compensation for acidosis	Respiratory failure	Monitor	Ongoing	A	Early sign	✓	Yes
Phase 1 Deterioration	SpO2 85%	Severe hypoxia	Immediate	Life threatening	Cardiac arrest	Apply O2	Reassess	A R	ABC priority	✓	Yes
Phase 1 Deterioration	Temp 38.1	Ongoing infection	Important	Supports sepsis	Delayed treatment	Give Tylenol	Reassess	B	Infection management		No

SEPTIC SHOCK Rou Ann Young

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Phase 1 Deterioration	Lethargic	Decreased LOC	Immediate	Organ dysfunction	Miss deterioration	Assess neuro	Frequent	A	LOC priority	✓	Yes
Phase 1 Deterioration	Weak pulses	Poor perfusion	Immediate	Shock sign	Organ failure	Fluid resuscitation	Reassess	A	Perfusion	✓	Yes
Phase 1 Deterioration	Sluggish cap refill	Poor circulation	Immediate	Shock indicator	Delayed recognition	Assess perfusion	Ongoing	A	Perfusion assessment	✓	Yes
Phase 1 Interventions	Oxygen administration	Improves oxygenation	Immediate	ABC priority	Hypoxia	Apply O2	Reassess	A R	Airway first	✓	Yes
Phase 1 Interventions	Fluid bolus LR	Restores perfusion	Immediate	Most important intervention	Shock progression	Administer fluids rapidly	Reassess BP HR	A R	Fluids first	✓	Yes
Phase 1 Interventions	Antibiotics (Vancomycin)	Treat infection	Important	Second priority after fluids	Sepsis progression	Administer antibiotics	Monitor	B	Timing matters	✓	Yes
Phase 1 Interventions	IV Tylenol	Reduce fever	Relevant	Comfort and metabolic demand	Minimal	Administer	Reassess	B	Supportive care		No
Phase 1 Interventions	Ondansetron optional	Symptom control	Relevant	Not priority	Minimal	Administer PRN	N/A	B	Low priority		No
Phase 1 Response	HR improves to 120	Shows response to fluids	Important	Indicator of improvement	Miss trend	Monitor	Ongoing	A	Trend improvement		No
Phase 1 Response	BP improves to 105/68	Improved perfusion	Immediate	Shows fluid effectiveness	Miss improvement	Trend BP	Ongoing	A	Reassessment critical	✓	Yes

SEPTIC SHOCK Rou Ann Young

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Phase 1 Response	SpO2 improves to 95% on O2	Improved oxygenation	Immediate	Shows intervention success	Miss decline	Monitor	Ongoing	A	Reassess always	✓	Yes
Phase 1 Response	Pain decreases to 3/10	Improvement overall	Relevant	Not priority	Minimal	Monitor	N/A	B	Comfort		No
Scenario-Wide Teaching	Fluids before antibiotics	Key sepsis principle	Immediate	Life-saving priority	Delayed treatment	Prioritize fluids	N/A	A	Priority thinking	✓	Yes
Scenario-Wide Teaching	Reassessment after intervention	Core nursing skill	Immediate	Ensures effectiveness	Miss deterioration	Reassess all interventions	Ongoing	A R	Reassessment is critical	✓	Yes
Scenario-Wide Teaching	Trend recognition	Vitals over time matter	Important	Detect deterioration early	Miss changes	Trend data	Ongoing	A	Clinical judgment	✓	Yes

Section	Debrief Question	Instructor Prompt
Reaction	What did you notice first about this patient?	Encourage recognition of abnormal vitals and overall instability
Reaction	At what point did you feel the patient was unstable?	Guide toward early recognition of shock cues
Analysis	What findings indicated this patient was in septic shock?	Focus on vitals lactate LOC perfusion
Analysis	Why are fluids prioritized before antibiotics?	Emphasize perfusion and shock physiology
Analysis	What cues showed poor perfusion?	Guide toward BP HR cap refill lactate
Analysis	How did lab values support your clinical judgment?	Encourage linking labs with assessment
Clinical Reasoning	What would you include in your SBAR?	Focus on concise critical communication
Clinical Reasoning	How did you prioritize your interventions?	Emphasize ABCs and fluids first
Application	What would you do differently next time?	Encourage reflection and improvement
Application	How will this impact your real clinical practice?	Connect to real patient care and early sepsis recognition

Septic Shock

Students are expected to recognize shock or MODS, initiate appropriate interventions, and reassess after treatment.

Critical Cues

- ☐ Hypotension
- ☐ Tachycardia
- ☐ Fever
- ☐ Altered mental status
- ☐ Poor perfusion

Expected Priorities

- ☐ Sepsis recognition
- ☐ Tissue perfusion
- ☐ Early escalation of care

Expected Interventions

- ☐ Oxygen
- ☐ IV fluids
- ☐ Antibiotics as ordered
- ☐ SBAR communication

Expected Reassessment

- ☐ Vital signs
- ☐ Urine output
- ☐ Mental status
- ☐ Response to treatment

Common Student Errors

- Delayed fluid resuscitation
- Focusing on antibiotics before stabilization
- Incomplete SBAR
- Failure to reassess

CJMM-ALIGNED SIMULATION DEBRIEFING
QUICK REFERENCE GUIDE

This guide provides flexible reflection prompts aligned with the NCSBN Clinical Judgment Measurement Model (CJMM). Select, adapt, and sequence questions based on simulation objectives, student performance, and learner needs. Use the prompts as a cognitive aid to support discussion and reflection, not as a required script or checklist.

1. Recognize Cues
- What patient findings or changes in condition stood out to you?
- Which assessment findings were most important?
- What changes suggested that the patient's condition was improving or worsening?
- Were there any important cues that were initially missed or overlooked?

2. Analyze Cues
- What did the assessment findings suggest was happening with the patient?
- How were the important cues related to one another?
- How did the patient's pathophysiology help explain the findings?
- What additional information would have helped you better understand the patient's condition?

3. Prioritize Hypotheses
- What did you believe was the patient's highest-priority problem?
- Which patient problem or concern required attention first, and why?
- What was the greatest immediate risk to the patient?
- How did you decide which problem was most urgent?

4. Generate Solutions
- What nursing interventions did you consider for the patient's priority problem?
- What outcome did you expect from those interventions?
- What other interventions or approaches could have been considered?
- What additional resources, information, or team members could have helped guide the plan of care?

5. Take Action
- What action did you take first, and why?
- How did the patient's condition influence the order of your interventions?
- Were there any actions that should have been taken earlier or differently?
- How did communication and teamwork affect the care provided?

6. Evaluate Outcomes
- How did you determine whether the patient responded to the interventions?
- What findings needed to be reassessed after the interventions?
- Did the patient's response require a change in the plan of care?
- What would you continue to monitor or reassess if care of this patient continued?

7. Reflection and Future Clinical Application
- What went well, and what challenged you the most?
- What would you do differently if you encountered a similar situation again?
- How will you apply what you learned from this experience to future patient care?